Which treatment this guide covers NCCP 00221a covers adults with locally advanced or metastatic NSCLC with an activating EGFR mutation who have not received an EGFR TKI. The separate licensed squamous-cancer indication after platinum treatment is not included in this NCCP regimen or its evidence card.

NCCP 00221 · VERSION 5

Afatinib monotherapy

The usual starting dose is 40 mg by mouth once daily, continuously without a planned rest week. Use the dose on your prescription. Treatment continues while it helps and side effects remain acceptable; any dose change is agreed with your team.

Oral targeted therapy · selected medicine-specific effects. Full details and sources are in the complete reference.

A picture of side effects

Numbered symptom boxes connect to the anatomical illustration. The medicine responsible may be uncertain; report the symptom.

1 New cough or breathlessness

afatinib

Possible lung inflammation needs urgent assessment.

2 Diarrhoea or severe abdominal pain

afatinib

Dehydration can injure the kidneys; severe abdominal pain may need emergency assessment.

3 Rash, nail changes or blistering

afatinib

Report early; severe blistering or peeling is urgent.

4 Eye pain or blurred vision

afatinib

Possible keratitis needs prompt specialist review.

5 Yellow eyes or skin

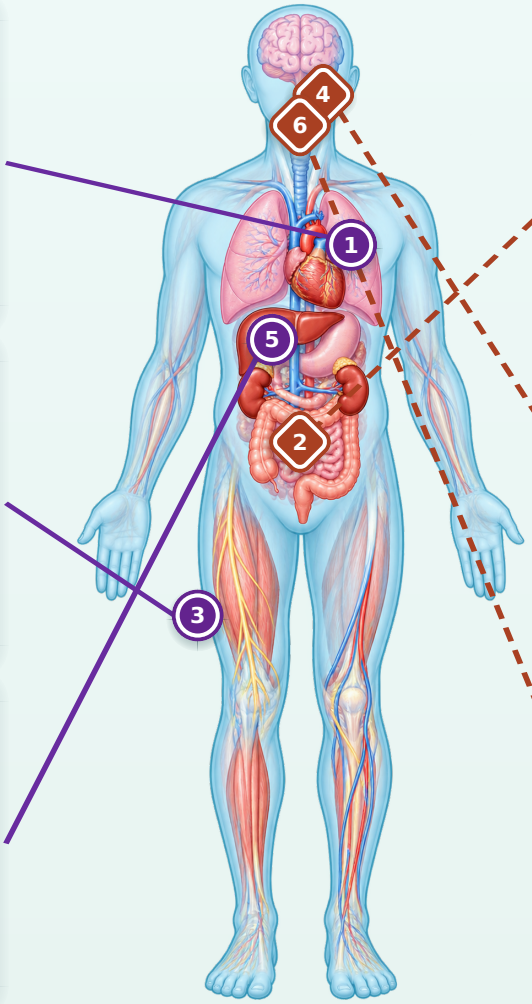
afatinib

Liver injury may need blood tests and treatment review.

6 Mouth soreness

afatinib

Support can help eating, drinking and mouth care.



Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Call urgently for new or worsening breathlessness, cough or fever; severe diarrhoea, inability to drink or much less urine; severe or persistent abdominal pain; blistering or peeling skin; or yellow skin/eyes. Eye pain, light sensitivity or blurred vision needs prompt assessment. Severe breathing difficulty, collapse or swelling of the tongue/throat needs emergency help. Use your unit’s temperature and 24-hour contact instructions. Severe chest pain, major breathing difficulty, collapse or stroke symptoms: 112 / 999.

Know the next step.

Taking treatment

The usual starting dose is 40 mg by mouth once daily, continuously without a planned rest week. Use the dose on your prescription. Treatment continues while it helps and side effects remain acceptable; any dose change is agreed with your team.

Take at the same time daily without food: leave at least 3 hours after food before the dose and wait at least 1 hour afterwards before eating. Swallow with water. If missed, take it the same day unless the next dose is due within 8 hours; then skip. Never double a dose or repeat a vomited dose. Ask pharmacy for the approved water-dispersion method if swallowing is difficult.

Checks and follow-up

Confirm the activating EGFR test, previous treatments and prescription. Check blood count, kidneys and liver. A heart-pumping test (ECHO or MUGA) is used if clinically indicated. Review lungs, bowel, eyes, skin, all medicines and pregnancy precautions. NCCP specifies blood count, kidney and liver tests monthly. Heart-function tests are repeated if clinically indicated. Report diarrhoea and skin changes early, especially in the first weeks. Scans and cancer reviews follow your Consultant's plan.

The whole-treatment map shows the checks for this exact treatment setting.

Support between reviews

Diarrhoea and fluids

Keep prescribed anti-diarrhoea medicine available from the first dose. Start your agreed plan at the first loose stool, keep drinking if able and report symptoms early. Your team may advise continuing symptom medicine until loose stools have stopped for 12 hours. Severe diarrhoea, weakness or reduced urine needs urgent advice.

Skin, nails and mouth

Use the agreed EGFR-inhibitor skin-care and mouth-care plan. Emollients and sun protection may help; report rash, painful nails or mouth sores early. The team decides whether creams, antibiotics or other treatment are needed.

Nausea medicines

Afatinib has minimal-to-low oral emetic risk. Routine infusion premedication is not required. Agree any as-needed anti-sickness medicine with the team, after checking interactions and your medical history.

Medicine interactions

Give pharmacy a list of all medicines, herbal products and supplements. Some P-gp inhibitors need separated dosing; strong inducers such as St John's wort can reduce afatinib exposure. Ask for a written individual schedule before adding or changing medicines.

Pregnancy, fertility and breastfeeding

Avoid pregnancy. Women who could become pregnant need adequate contraception during treatment and for at least 1 month after the final dose. Do not breastfeed during treatment. Discuss fertility and future pregnancy plans before starting.

Oncology daytime: Insert numbers here. **Out of hours:** Insert numbers here. Follow your own hospital's urgent-care plan. Emergency: 112 / 999.



Your complete treatment reference

https://sactcheck.com/patient/00221/

Public information only. No personal information is encoded.